

PREFACE

Why This Book Exists

American healthcare is the most expensive, most administratively complex, and most inequitably distributed healthcare system in the developed world. This is not a new observation. It has been documented, analyzed, debated, and lamented for decades. What is new — or more precisely, what is newly urgent — is that the system is now visibly breaking under pressures that cannot be managed through incremental adjustment. The financial logic of fee-for-service payment, the demographic reality of an aging population, the exhaustion of a clinical workforce buried in administrative burden, and the structural inadequacy of a hospital network designed for a different century are converging into a crisis that requires systematic response, not marginal reform.

This book is a response to that crisis. It is designed for the healthcare leaders, policymakers, analysts, and students who must understand not just what is wrong with American healthcare but what a transformed system would look like, how to get there, and what the organizations executing that transformation need to know to succeed. It is simultaneously an academic text — grounded in evidence, rigorous in its analytical framework, honest about what is uncertain — and a practitioner's guide — organized around decisions, investments, contracts, and programs that real organizations must make in real time.

The book is organized around the Health Transformation Review six-pillar framework: Policy, Economics, Technology, Clinical, Equity, and Operations. This framework emerged from the conviction that healthcare transformation fails when it addresses any single pillar in isolation. Payment reform without clinical redesign produces financial savings that evaporate when care models do not change. Clinical quality improvement without payment reform produces better care that the financial system immediately discourages. Technology investment without operational readiness produces platforms that no one uses. Equity goals without structural SDOH investment produce aspirations without outcomes. The six pillars must move together — which means the people responsible for each pillar must understand all the others.

Vermont runs through this book as its primary teaching case. This requires explanation, because Vermont is a small state — 647,000 people, 14 hospitals, roughly the population of a mid-sized American city — and its direct policy relevance might seem limited. Vermont is in this book not because Vermont is large or economically powerful, but because Vermont is early. Early in its demographic aging. Early in its hospital financial fragility. Early in its recognition that voluntary payment reform cannot produce the structural change a system in crisis requires. And early in its willingness to legislate the structural responses — Acts 167 and 68, the AHEAD Model participation, the Rural Health Transformation Program — that other states will reach for when their own systems arrive at Vermont's current condition.

Vermont is also unusually transparent. The combination of a small state's intimacy, a strong public-sector analytical tradition, and an active regulatory body in the Green Mountain Care Board has produced a public record of healthcare system analysis — the Oliver Wyman Act 167 report, the GMCB's monthly hospital data, the AHS transformation reports, the AHEAD State Agreement — that gives this book a documentary foundation most states cannot match. Vermont's healthcare transformation is happening in public, with detailed data, and the lessons are directly transferable.

A word about what this book is not. It is not a policy brief for or against any particular political agenda. Healthcare transformation is contested terrain, and this book presents the evidence and the analytical frameworks that inform decisions without pretending those decisions are purely technical. The five goals of Vermont's Act 167 — reduce

inefficiencies, lower costs, improve health outcomes, reduce health inequities, and increase access to essential services — are goals that virtually every person across the political spectrum would endorse. The means for achieving them are legitimately contested, and this book engages that contestation honestly.

It is also not a prediction. The transformations described in this book are underway but unfinished. The outcome of Vermont's experiment — whether the hospital system stabilizes or contracts, whether global budgets reduce costs or constrain access, whether the Agency of Human Services can execute the restructuring its mandate requires — is genuinely uncertain. This book provides the analytical framework for understanding what is happening and why, and for contributing to better decisions as the story unfolds. The decisions, and their consequences, belong to the people who make them.

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INTRODUCTION

The Transformation Imperative

A System at the Breaking Point

On September 18, 2024, the Green Mountain Care Board of Vermont convened a public meeting at which Oliver Wyman Healthcare and Life Sciences Group presented the findings of a year-long, \$1 million statewide hospital systems analysis commissioned under Act 167 of 2022. The room was full. The findings were not surprising to anyone who had been paying attention — but they were stated with a precision and a comprehensiveness that made them impossible to dismiss.

Nine of Vermont's fourteen hospitals were reporting operating losses in 2023, with the worst reaching negative 8.9%. If current trends continued, thirteen of the fourteen would be reporting losses by 2028. The cumulative five-year deficit facing the system ranged from \$700 million against break-even under conservative assumptions to \$2.4 billion against break-even under realistic ones. Average premiums for silver exchange plans had increased 108% in six years, while median household income had grown 22%. Out-of-pocket maximums had more than doubled in five years. The working-age population was declining; the over-65 population was growing toward 30% of the state's total by 2040.

The Oliver Wyman consultants offered a three-imperative summary of what Vermont needed to do: build housing and fix transportation; pay hospitals with reference-based pricing and move to global budgets; and move all care possible out of hospitals. These were not incremental recommendations. They were structural prescriptions for a system that, absent structural change, was heading toward collapse.

Vermont is not an outlier. It is a preview.

The structural forces producing Vermont's crisis — demographic aging, hospital financial fragility, the unsustainability of commercial cross-subsidy, the failure of voluntary payment reform, the exhaustion of a workforce buried in administrative complexity — are present to varying degrees in every American state. Vermont faces them earlier and more acutely because of its demographics and its economic structure. But the states of the Northeast, the rural Midwest, and eventually the national system will arrive at Vermont's current condition on a lag of ten to twenty years. What Vermont is forced to solve now, the rest of the country will be forced to solve later.

This book is written in that context. It is not a retrospective analysis of a completed transformation. It is a real-time account of a transformation in progress, with all the uncertainty, difficulty, and instructiveness that implies.

What Transformation Actually Means

The word 'transformation' is used so frequently in healthcare that it has nearly lost its meaning. Press releases describe routine process improvements as transformational. Strategic plans invoke transformation as aspiration rather than commitment. This book uses the word in its precise sense: transformation is structural change that alters the fundamental incentives, relationships, and capabilities of a system, producing different behaviors and different outcomes rather than incremental improvements within the existing structure.

By this definition, most of what has been called healthcare transformation over the past fifteen years has not been transformational. Voluntary accountable care organization programs that high-cost providers can decline to join are not transformation. Shared savings models that reward hospitals for reductions in utilization they would have

achieved anyway are not transformation. Electronic health record adoption that digitizes paper processes without changing care delivery is not transformation. Quality measurement initiatives that produce reports without affecting payment are not transformation.

Transformation, in the sense this book uses the term, looks like what Vermont is attempting: mandatory reference-based pricing that eliminates the ability of dominant hospitals to charge commercial payers three to four times Medicare rates; statutory global hospital budgets that change every hospital's financial incentive from maximizing volume to managing population health; a Statewide Health Care Delivery Strategic Plan that commits Vermont's Agency of Human Services to specific, measurable outcomes by specific dates; and capital investment through the Rural Health Transformation Program that builds the community infrastructure that the payment reform requires to produce population health improvement rather than just cost reduction.

Whether this constitutes successful transformation will be determined by what happens over the next five to ten years. What is already clear is that Vermont has moved further toward genuine structural change than any comparably sized system in recent American health policy history.

The Six-Pillar Framework: Why All Six Must Move Together

The central analytical contribution of this book is the six-pillar framework for healthcare transformation: Policy, Economics, Technology, Clinical, Equity, and Operations. The framework is not a checklist of things that should be done. It is an integrated analytical architecture for understanding why transformation attempts fail when they address any pillar in isolation.

The most common failure mode in healthcare transformation is single-pillar thinking. A payer that focuses exclusively on the Economics pillar — redesigning payment models — while leaving the Technology, Clinical, and Operations pillars unchanged produces payment reform that providers cannot operationalize. Hospitals without population health analytics infrastructure cannot manage global budgets. Clinicians without team-based care models cannot deliver the primary care access that makes reduced hospitalization possible. Administrators without administrative simplification cannot absorb the management complexity of value-based contracts on top of fee-for-service billing requirements.

Conversely, a technology vendor that focuses exclusively on the Technology pillar — building sophisticated health information exchange and population health platforms — while leaving the Economics and Operations pillars unchanged builds tools that no one has the financial incentive or organizational capacity to use. Vermont's Oliver Wyman analysis found this pattern explicitly: VITL (Vermont's health information exchange) exists but is not widely used, OneCare (Vermont's ACO) exists but participation is voluntary and incomplete, and the 'all-payer' system does not actually include all payers. The infrastructure exists; the incentives and operations to use it effectively do not.

The six pillars interact through a logic that this book develops in detail in Chapter 1. The short version is this: payment reform (Economics pillar) changes what behaviors the financial system rewards; clinical redesign (Clinical pillar) changes how care is actually delivered; technology investment (Technology pillar) provides the data infrastructure that makes new care models manageable at scale; policy and regulatory change (Policy pillar) creates the mandatory architecture that ensures voluntary actors cannot opt out of structural reform; equity investment (Equity pillar) ensures that transformation benefits reach the populations that need them most and does not concentrate gains among those already well-served; and operational execution (Operations pillar) converts strategy into implemented programs, trained staff, and measured outcomes.

Remove any one pillar and the transformation fails. Vermont's reform agenda is instructive precisely because Acts 167 and 68, the AHEAD Model, and the Rural Health Transformation Program together address all six pillars simultaneously — imperfectly

and incompletely, as any real-world policy effort does, but with a comprehensiveness that distinguishes Vermont's approach from single-pillar interventions.

How to Use This Book

This book is organized in three parts. Part I (Chapters 1-3) establishes the foundational framework: the six-pillar architecture, the Vermont system portrait, and the analytical tools for reading transformation efforts in any context. Part II (Chapters 4-15) develops each pillar in depth, with a dedicated chapter for each, integrating academic framework, evidence base, Vermont case study, and practical application for healthcare leaders and policymakers. Part III (Chapters 16-17) addresses advisory practice and the future — the institutional infrastructure for translating analysis into action, and the forces shaping American healthcare through 2035.

Throughout Part II, Vermont functions as a continuous teaching case rather than an isolated example. Each chapter begins with the national analytical framework for its pillar, develops the evidence base from national and international research, and then grounds the framework in Vermont's specific implementation — using Vermont's acts, Oliver Wyman's findings, GMCB's data, and AHS's transformation reports as the primary case material. This structure allows a reader who is not specifically interested in Vermont to use the national framework; a reader focused on Vermont to use the Vermont sections as a field guide; and a reader designing transformation efforts elsewhere to use Vermont as a template and a cautionary inventory of what is hard.

The book concludes with a chapter specifically addressed to the Vermont Agency of Human Services and the Health Care Delivery Advisory Committee: a six-pillar roadmap for the Statewide Health Care Delivery Strategic Plan that AHS must deliver to the Vermont legislature by December 2028. This chapter is simultaneously the most specific and the most generalizable section of the book — because the question it addresses (how does a state agency restructure itself to serve as a population health system operator?) is the question every state will face as their healthcare systems reach the condition Vermont is managing today.

CHAPTER ONE

The Six-Pillar Framework

Chapter 1 establishes the six-pillar framework that organizes this entire book. It answers three questions: what each pillar is, how the pillars interact with each other, and why transformation fails when any pillar is neglected. It also introduces the Vermont thread that runs through every subsequent chapter — establishing Vermont’s system in broad strokes before Chapters 2 and 3 develop the detail.

The Problem with Single-Pillar Thinking

In 2010, the Affordable Care Act created the Center for Medicare and Medicaid Innovation with a mandate to test new payment and delivery models and scale what worked. Over the following fifteen years, CMMI launched more than fifty models, investing billions of dollars and the attention of thousands of healthcare organizations. The results were modest. A 2023 evaluation found that the vast majority of CMMI models had not achieved statistically significant reductions in spending or improvements in quality. Most were discontinued. A handful showed promise and were expanded.

The pattern of failure across CMMI models is instructive. Models that addressed only the Economics pillar — changing what Medicare paid without addressing how care was delivered or whether providers had the infrastructure to respond — generated financial pressure without behavioral change. Models that addressed only the Clinical pillar — redesigning care coordination or care management programs — produced clinical improvements that the payment system then penalized by reducing volume-based revenue. Models that required Technology pillar investment — electronic health records, population health platforms, data exchange — found that providers lacked the operational capacity to use the tools effectively.

The lesson is not that payment reform, clinical redesign, or technology investment is wrong. It is that each intervention, pursued in isolation, runs into the constraints created by the pillars it ignores. This is the core analytical argument of the six-pillar framework: healthcare transformation is a system problem, and system problems require system solutions.

The Six Pillars Defined

The six pillars are not equal in their causal priority — payment reform (Economics) and enabling legislation (Policy) are preconditions that the other pillars depend on — but they are equal in their necessity. A transformed healthcare system requires all six to be functional.

1 Policy	The legislative and regulatory environment that creates the mandatory architecture for transformation. Policy establishes what actors must do, what they are prohibited from doing, and what governance structures ensure accountability. Voluntary reform operates within the Policy pillar's constraints; structural transformation requires it to change. Vermont application: Acts 167 and 68, the AHEAD Model State Agreement, and the Statewide Strategic Plan mandate are Vermont's Policy pillar interventions. Each converts voluntary reform into statutory obligation.
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2 Economics	The payment and financial architecture that determines what behaviors the system rewards. The Economics pillar encompasses payment reform (from fee-for-service to value-based and global payment), financial modeling, total cost of care management, and the hospital financial sustainability that transformation requires. Vermont application: <i>Reference-based pricing (FY2027), global hospital budgets (FY2028-2030), and the AHEAD Model's total cost of care accountability are Vermont's Economics pillar interventions.</i>
3 Technology	The data infrastructure, health information exchange, analytical platforms, and clinical technologies that make population health management possible at scale. The Technology pillar is the information substrate on which all other pillar functions depend — you cannot manage what you cannot measure. Vermont application: <i>VHCURES (all-payer claims database), VITL/VHIE (health information exchange), the AHS-GMCB analytics vendor, AI scribe grants, and Vermont's Clinically Integrated Network are Technology pillar investments.</i>
4 Clinical	The care delivery models, quality measurement frameworks, clinical workforce design, and patient care programs that determine how care is actually delivered. The Clinical pillar is where payment reform becomes patient outcomes — or fails to. Vermont application: <i>The Blueprint for Health (all-payer PCMH program), Collaborative Care Model behavioral health integration, CCBHC expansion, and the primary care investment strategy are Vermont's Clinical pillar interventions.</i>
5 Equity	The programs, investments, and accountability structures that ensure transformation benefits reach all populations — and specifically those most harmed by the current system's failures. Equity is not a soft afterthought; it is an analytical requirement. A transformation that reduces average costs while widening disparities has failed. Vermont application: <i>Geographic equity across Vermont's 14 HSAs, SDOH investment through the EAST Fund and Blueprint HRSN screening, rural access preservation in the RBP and global budget design, and health equity metrics in Act 167/68 reporting are Vermont's Equity pillar interventions.</i>
6 Operations	The organizational capacity, management infrastructure, administrative simplification, and implementation discipline that converts strategy into executed programs. The Operations pillar is where transformation plans become real or remain documents — the graveyard of well-designed reforms that could not be implemented. Vermont application: <i>The 14-hospital transformation planning process, RHRC technical assistance, the AHS HSA Coordinator model, the Division of Planning and Effectiveness, and EMS regionalization are Vermont's Operations pillar investments.</i>

How the Pillars Interact: The Transformation Logic

The six pillars do not operate independently. They interact through a set of dependencies and feedback loops that determine whether transformation produces the outcomes it promises. Understanding these interactions is the core analytical skill this book develops.

The foundational dependency is between Policy and Economics. Payment reform (Economics) requires policy authority — either statutory mandate or regulatory power — to be effective at scale. Vermont learned this lesson through a decade of voluntary payment reform efforts: the Vermont All-Payer ACO Model, OneCare's voluntary hospital participation, VITL's voluntary HIE connectivity. Each produced partial results

because voluntary participation allowed the highest-cost actors to opt out. Act 68's mandatory RBP and global budget requirements are a Policy pillar response to the failure of Economics pillar interventions without Policy pillar backing.

The second critical dependency is between Economics and Clinical. Payment reform changes the financial incentive structure; clinical redesign changes the care delivery model to respond to the new incentives. A hospital operating under a global budget has a financial incentive to prevent hospitalizations — but it can only act on that incentive if it has the primary care network, care coordination infrastructure, and behavioral health integration to actually keep patients out of the hospital. Without the Clinical pillar investment, the Economics pillar incentive produces financial pressure without clinical improvement.

The third dependency is between Technology and all other pillars. You cannot attribute patients to hospitals for global budget purposes without accurate patient matching across providers. You cannot identify care gaps for quality improvement without population-level claims and clinical data. You cannot model the financial impact of service line closures without analytics infrastructure. You cannot monitor equity metrics without data stratified by race, income, and geography. Technology is the information substrate on which every other pillar's management functions depend.

The Equity pillar has a different kind of dependency: it is simultaneously a constraint on every other pillar and a beneficiary of all of them. Every payment reform intervention, clinical redesign, and technology investment must be evaluated for its equity implications — does it reduce disparities or exacerbate them? Does it reach the populations that need it most or concentrate benefits among those already well-served? Vermont's Act 168 goals explicitly include reducing health inequities alongside reducing costs and improving outcomes, recognizing that transformation that achieves the first two without the third is incomplete.

The Operations pillar is the translation layer — the organizational capacity that converts policy intent, payment incentives, technology tools, clinical models, and equity goals into implemented programs with trained staff and measured outcomes. A transformation strategy that is analytically sound, financially compelling, technically feasible, and clinically validated will still fail if the organization responsible for executing it lacks the project management infrastructure, the change management capability, and the sustained leadership attention that execution requires.

The Vermont Thread: Six Pillars in One System

Vermont's healthcare transformation is the closest thing to a controlled experiment in simultaneous six-pillar intervention that American health policy has produced. The following table maps Vermont's primary reform vehicles onto the six-pillar framework, establishing the connective tissue that runs through every subsequent chapter.

Pillar	Primary Vermont intervention	Secondary interventions	Chapter
Policy	Acts 167 (2022), 51 (2023), 68 (2025): the legislative reform cascade; AHEAD Model State Agreement (January 2025)	Act 55 (drug pricing); Act 49 (insurer solvency); Act 62 (HIE governance); federal RHT Program award	Chapter 4
Economics	Reference-based pricing (mandatory, FY2027); global hospital budgets (FY2028-2030); AHEAD total cost of care accountability	Act 68 hospital spending reduction (2.5%, FY2026); GMCB FY27 budget guidance (-1% commercial rate); Act 55 drug cap (\$135M savings)	Chapter 6
Technology	VHCURES all-payer claims database; Vermont	VITL/VHIE HIE; DVHA HIT Plan (Act 62); statewide EHR	Chapter 14

	CIN (RHT Program); AHS-GMCB analytics vendor; AI scribe and RPM grants	feasibility assessment; FHIR compliance investments	
Clinical	Blueprint for Health (all-payer PCMH + CHT); CoCM and MHI behavioral health integration; CCBHC expansion (7 entities by 2026)	Hub and Spoke SUD model; PACE program development; COE regionalization; dementia care infrastructure	Chapter 10
Equity	Geographic equity monitoring (HSA-level); SDOH integration through AHS HSA Coordinators; rural access preservation in RBP/global budget design	HRSN universal screening (Blueprint); CCBHC access for all populations; Northeast Kingdom focus; GLP-1 access advocacy	Chapter 12
Operations	14-hospital transformation planning (RHRC + AHS); Vermont CIN shared services; Act 68 transformation grants (\$2M); PMO establishment	EMS regionalization; administrative simplification (CON, prior auth); AHS restructuring; Division of Planning and Effectiveness	Chapters 8, 11

Figure 1.1 — Vermont's six-pillar transformation map. Sources: Acts 167, 51, 55, 62, 68 of 2022-2025; AHEAD State Agreement; Vermont RHT Program Application; Oliver Wyman Act 167 Report.

This table is not a comprehensive inventory of Vermont's healthcare reform. It is a navigation map — a way of locating Vermont's specific interventions within the analytical framework that the subsequent chapters develop. A reader working through Chapter 4 (Policy pillar) will recognize Acts 167 and 68 as the Policy pillar's legislative expression. A reader in Chapter 6 (Economics pillar) will see reference-based pricing and global budgets as the Economics pillar's operative mechanisms. The framework gives every Vermont policy intervention a structural home and every analytical concept a Vermont application.

Key Concepts in This Chapter

Term	Definition
Six-pillar framework	The HTR analytical architecture organizing healthcare transformation across six interdependent domains: Policy, Economics, Technology, Clinical, Equity, and Operations. Transformation requires all six pillars to be functional; failure in any one pillar undermines the others.
Single-pillar thinking	The failure mode in which a transformation initiative addresses one pillar — typically payment reform or clinical redesign — without attending to the constraints and dependencies created by the other five pillars.
Reform cascade	A legislatively structured sequence in which each reform intervention builds on the institutional capacity and political legitimacy created by its predecessor. Vermont's Act 167 → Act 51 → Act 68 sequence is the prototype.
Structural transformation	Change that alters the fundamental incentives, relationships, and capabilities of a system — as distinguished from incremental improvement within the existing structure. This book uses 'transformation' in this precise sense throughout.

Vermont as preview	The analytical premise that Vermont's healthcare challenges — demographic aging, hospital financial fragility, commercial cross-subsidy unsustainability, voluntary reform failure — are present nationally on a 10-20 year lag, making Vermont's responses a template for the nation's future policy toolkit.
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CHAPTER TWO

The Vermont System Portrait

Chapter 2 develops the Vermont system portrait — the demographic, financial, clinical, and structural facts about Vermont's healthcare system that every reader needs before the reform narrative of subsequent chapters becomes fully legible. It is intentionally data-dense because the data tells a story that no amount of narrative can substitute for.

Vermont: The State

Vermont is the second-smallest state by population in the United States. Its 647,000 residents live across 14 counties and 255 towns and cities in a predominantly rural landscape — 9,616 square miles of mountains, valleys, farms, and forest. The population density is 70 people per square mile, well below the national average of 94. Nearly two-thirds of Vermont residents live in areas classified as rural by the federal definition used for healthcare program eligibility.

Vermont's geography is not merely a demographic fact. It is a healthcare infrastructure constraint. A patient in Essex County — one of the most rural and least populated counties in the Northeast Kingdom, with an uninsurance rate of 8% compared to Vermont's statewide 3% — may be 45 minutes from the nearest hospital emergency department, 90 minutes from a specialist, and hours from a tertiary care center. Public transportation is limited or unreliable in most rural counties. Broadband penetration reaches 80% of rural areas, leaving 20% without the connectivity that telehealth and remote patient monitoring require.

Vermont's economy is diverse but small. Health care is one of the most common industries in the state — employing approximately 14% of the workforce and representing a major economic anchor in rural communities where the hospital is often the largest employer. The University of Vermont Medical Center in Burlington, with approximately 50% of the state's hospital market share, is the economic center of gravity of Vermont's healthcare system. Every reform that affects UVMMC affects the Vermont economy in ways that hospitals in larger, more economically diverse states do not.

The Demographic Crisis in Data

Vermont's demographic trajectory is the foundational fact of its healthcare transformation challenge. Oliver Wyman's population projections, derived from MPR Vermont population data, tell the story with precision:

Vermont population aged 65+ by 2040 30%+ Up from 21.7% in 2020 — a 57% increase in the elderly population	Working-age population change by 2040 -13% From 367,000 to approximately 318,000 — the commercial insurance base shrinks
Total Vermont population 2020-2040 Flat Approximately 612,000-624,000 throughout — aging, not growing	Vermont residents with health insurance (2025) 97%

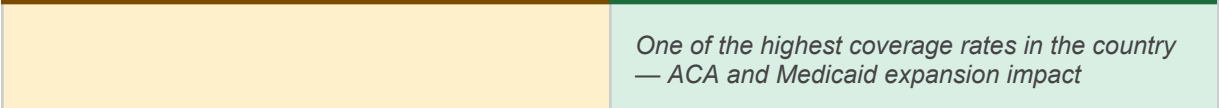


Figure 2.1 — Vermont demographic transformation: key projections. Sources: Oliver Wyman Act 167 Report (2024); Vermont RHT Program Application; KFF.

The implications of these demographic trends are not complicated — they are simply severe. An aging population means a shift in the payer mix toward Medicare and Medicaid, both of which reimburse hospitals below cost. A shrinking working-age population means a shrinking commercially insured population, reducing the cross-subsidy that currently sustains hospital operations. More complex care needs among the elderly means higher costs per patient. And an aging healthcare workforce — Vermont has the same age distribution among its clinicians as among its population generally — means accelerating retirements of experienced providers in exactly the years when demand is rising.

Oliver Wyman's analysis named this dynamic directly: the cross-subsidy model that has sustained Vermont's rural hospital system is becoming structurally impossible to maintain as demographics shift. The commercial premiums that make it possible for hospitals to accept below-cost Medicare and Medicaid payments depend on a sufficient pool of commercially insured patients. As that pool shrinks relative to the Medicare and Medicaid population, the math stops working — not in a distant future scenario, but on the trajectory Vermont is already on.

The Fourteen Hospitals

Vermont has 14 general acute care hospitals, all of which are non-profit. This fact — no for-profit hospitals — is significant for the reform agenda: Vermont's hospital system does not have the shareholder value obligations that complicate transformation discussions in states with investor-owned hospital chains. Vermont's hospitals are community assets, governed by community boards, with legal missions oriented toward community health. This creates better conditions for the kind of collaborative transformation Vermont is attempting — but it does not eliminate the financial pressures, institutional interests, or organizational inertia that make transformation difficult everywhere.

The 14 hospitals divide into three functional categories that correspond directly to the Oliver Wyman regionalization blueprint and the tiered network model developed in Chapter 8:

Category	Hospitals	Financial position (FY2023)	Role in transformed system
Large PPS hospitals (Prospective Payment System)	UVMMC (Burlington); Rutland Regional; SW Vermont Medical Center; Brattleboro Memorial; Central VT Medical Center; NW Medical Center; NE VT Regional Hospital	Mixed — UVMMC reported positive operating margins in most years; community PPS hospitals showed operating losses of -3% to -8.9% in 2023	Regional Specialty Centers (RSCs) or Tier 1; designated Centers of Excellence in multiple specialties; referral destinations for the network
Critical Access Hospitals (CAHs)	Copley (Morrisville); Gifford (Randolph); Grace Cottage; Mt. Ascutney;	Operating profit of \$938 per adjusted discharge vs. \$2,784 benchmark — the	Tier 2 or Tier 3; focused service scope; some candidates for

	North Country; Porter; Springfield Hospital	most financially distressed category	REH or CACC conversion; protected by AHEAD and CAH-specific regulatory treatment
Network-affiliated mid-size hospitals	Porter (Middleton affiliated with UVM network); NW Medical Center; others with system affiliations	Variable; network affiliations provide some operational and capital support but do not resolve underlying financial pressures	COE-designated specialties within regional network; transition planning with AHS support

Figure 2.2 — Vermont's 14-hospital categorization by type, financial position, and transformed system role. Sources: Oliver Wyman Act 167 Report; AHS November 2025 Transformation Report; GMCB hospital financial data.

The distribution of financial distress across these categories matters for reform design. CAHs — the smallest, most rural hospitals serving the communities with the fewest alternatives — are the most financially distressed. They are also the hospitals where service reductions or closures would have the most severe community consequences, and the hospitals that the AHEAD Model's global budget design must protect most carefully. Any transformation strategy that solves Vermont's hospital financial crisis by reducing services in the Northeast Kingdom without replacing them with adequately resourced alternatives has not transformed the system — it has abandoned its most vulnerable communities.

The Insurance and Premium Crisis

Vermont's health insurance market is dominated by Blue Cross Blue Shield of Vermont and MVP Health Care. Vermont was an early adopter of the ACA, expanding Medicaid in 2014 and establishing Vermont Health Connect, the state's insurance exchange. The result is a 97% coverage rate — one of the highest in the country — that is a genuine policy achievement and a source of legitimate state pride.

The achievement of near-universal coverage has not, however, resolved Vermont's affordability crisis. Coverage is not the same as affordability. Vermont's individual market monthly premium rose from \$456 in 2018 to \$948 in 2024 — a 108% increase — while median household income grew 22%. Out-of-pocket maximums more than doubled. The insured Vermonter who delays necessary care because of the cost of their deductible or co-pay is not a healthcare access success story; they are an indictment of a system that has achieved coverage on paper while allowing costs to rise beyond reach.

The mechanism connecting hospital prices to insurance premiums is not complicated, but it is often presented as more mysterious than it is. Vermont hospitals charge commercial insurers 250-300% of Medicare rates on average for hospital services. In the Burlington area, UVMMC charged approximately 417% of Medicare for outpatient services in 2022. These prices — which bear no relationship to quality — translate directly into premiums. When a hospital charges 417% of Medicare for outpatient imaging, every insurer that covers that hospital's patients must charge premiums high enough to pay those prices and still maintain solvency. The result is the premium trajectory Vermont has experienced: driven primarily by hospital price increases that have no regulatory brake.

This is what reference-based pricing is designed to address. This is why it was Oliver Wyman's second imperative. And this is why Act 68's mandate to implement RBP and then global budgets — not as a demonstration or a pilot, but as statewide mandatory regulation — represents the most consequential health policy legislation Vermont has passed since Act 48 created the Green Mountain Care Board in 2011.

The Healthcare Workforce

Vermont's healthcare workforce faces a triple constraint that no single intervention can resolve: insufficient supply of primary care physicians, specialist shortage that cannot be addressed by adding more specialists to each community, and a housing and economic environment that makes recruiting and retaining clinical staff increasingly difficult.

The primary care shortage is the most immediately actionable. Vermont's RHT Program application projects a shortage of 370 full-time equivalent primary care providers by 2030 — 112 in family medicine and 190 in other primary care specialties. Oliver Wyman's analysis offered a counterintuitive but important qualification: HRSA recognizes no Health Profession Shortage Areas in Vermont. The problem is not purely one of physician headcount — it is partly one of primary care productivity and care model. If primary care providers were supported to see three patients per hour in team-based care settings where clinical staff work at the top of their licensure and administrative burden is minimized, Vermont would have adequate primary care supply for well into the future.

The specialist shortage requires a different solution: regionalization. Vermont cannot maintain cardiologists, neurologists, radiation oncologists, and orthopedic surgeons at every one of its 14 hospitals. The patient volume at smaller community hospitals does not justify the positions, and the clinical volume needed to maintain specialist expertise cannot be generated at a hospital serving 30,000-50,000 people. Oliver Wyman's Center of Excellence framework — concentrating specialty services at facilities with sufficient volume — is the correct response. The key is ensuring that concentration does not mean inaccessibility: telehealth consultation, reliable EMS transport, and clear referral pathways are the operational complements to regionalization.

The housing constraint is the most intractable and the most important to name honestly. Vermont's rental vacancy rate of 3% is well below the 5% that characterizes a healthy rental market. Half of Vermont renters are cost-burdened. Vermont has the second-highest per-capita homelessness rate in the country. Healthcare providers recruited to rural Vermont communities — particularly those from outside the state — face a housing market that may make it impossible to find an affordable place to live near the hospital where they would work. Oliver Wyman's first imperative — build housing and fix transportation — is a healthcare workforce policy as much as a social policy.

CHAPTER THREE

Reading This Book — A Guide for Different Audiences

Chapter 3 is the user's guide to the book — explaining how different readers can navigate the material, how the Vermont thread connects across chapters, how the HTR platform tools relate to the analytical frameworks, and what the AHS restructuring chapter is designed to do for the people who will actually use it.

The Four Reader Profiles

This book has four primary reader profiles. Each has a different relationship to the material and a different way of getting maximum value from the book.

The Healthcare Policy Professional

You work in a government agency, a legislative office, a think tank, or a foundation focused on healthcare policy. You have substantive policy knowledge but may not have deep familiarity with the operational mechanics of healthcare transformation — what it actually takes to implement a global budget, build a community health team, or manage a 14-hospital transformation planning process.

Recommended reading path: Read the Preface and Introduction in full, then Chapter 1 (six-pillar framework) and Chapter 2 (Vermont portrait). For each pillar you care most about, read the corresponding chapter in depth. Read Chapter 11 (AHS Restructuring) carefully — it is the most direct bridge between policy mandate and organizational design. Read Chapter 17 (future forecast) for the national context.

The Healthcare Executive or Administrator

You lead a hospital, health system, health plan, ACO, or healthcare services organization. You are making decisions — about contracts, capital investments, workforce, technology — that the transformation environment is complicating. You need the analytical framework but you especially need the operational implications.

Recommended reading path: Read the Introduction for the overarching argument, then go directly to the chapters most relevant to your immediate decisions. If you are navigating a VBC contract, Chapter 6 (Economics) is your entry point. If you are building care management programs, Chapter 10 (Clinical). If you are evaluating technology investments, Chapter 14 (Technology). Read Chapter 8 (Operations) for the hospital transformation planning framework. Return to Chapters 1-3 for the integrating logic.

The Vermont Practitioner

You work for AHS, GMCB, DVHA, the Vermont legislature, a Vermont hospital, a Blueprint practice, or a Vermont community organization. You are living inside the transformation this book describes. You need the national analytical context but you especially need the Vermont-specific operational guidance.

Recommended reading path: Read Chapter 2 (Vermont portrait) and Chapter 4 (Policy — Acts 167 and 68) first, since these are the chapters most directly grounded in Vermont's current situation. Then read Chapter 11 (AHS Restructuring) as the operational framework for the work you are doing. Use the implementation timeline tables throughout Part II as a reference guide to where Vermont is in each pillar's trajectory. The Vermont in Practice sidebars throughout are written for you specifically.

The Healthcare Student or Researcher

You are building foundational knowledge of healthcare transformation, either in a graduate program or as an independent researcher. You need the analytical architecture, the evidence base, and the theoretical frameworks, not just the Vermont case study.

Recommended reading path: Read the entire book in chapter order. Chapter 1 establishes the framework; Chapters 4-15 develop each pillar with full academic grounding before the Vermont application; Chapters 16-17 provide the institutional and future context. The Key Concepts sections at the end of each chapter are the glossary; the source notes are the bibliography. Use Vermont as the case study while developing transferable analytical skills.

The Vermont Thread: How It Runs Through the Book

Vermont appears in this book in four distinct registers, and recognizing which register you are reading helps calibrate how to use the material.

- Vermont as diagnostic case: The Oliver Wyman analysis of Vermont's hospital financial crisis, the GMCB price data, and the AHS demographic projections establish Vermont as an instance of a generalizable pattern of rural healthcare system stress. When these appear, they are teaching the pattern, not just Vermont's version of it.
- Vermont as legislative laboratory: Acts 167, 51, 68, and 62 are specific legislative interventions in a specific state, but their design logic — the reform cascade structure, the mandatory versus voluntary distinction, the statutory accountability metrics — is generalizable to any state undertaking comparable reform.
- Vermont as operational field report: The AHS transformation reports, the RHRC engagement methodology, the 14-hospital planning process, and the Health Care Delivery Advisory Committee's work are real-time accounts of a transformation in progress. They convey the texture of execution — the difficulties, the adaptations, the gap between strategy and implementation — that no policy analysis can substitute for.
- Vermont as national template: Chapter 11 (AHS Restructuring) and Chapter 17 (The Future) explicitly use Vermont as a template for the national condition. Here the argument is prospective: if Vermont succeeds, this is what success looks like; if Vermont faces these organizational design challenges, every state will face them; if Vermont's approach to the AHS restructuring mandate works, other states can adapt it.

The HTR Platform and This Book

This book is part of the Health Transformation Review ecosystem. The HTR platform — its intelligence feed, Research Lab tools, Academy curriculum, and Advisory services — provides the operational complement to the analytical framework the book establishes. Understanding the relationship between the book and the platform helps readers get maximum value from both.

The book provides the analytical framework: the six-pillar architecture, the evidence base for each pillar's mechanisms, the Vermont case study, and the conceptual vocabulary for understanding transformation. The HTR platform provides the operational tools: the APM Shared Savings Calculator that applies the Economics pillar's financial modeling to specific contracts; the Clinical Quality Optimizer that implements the Clinical pillar's HEDIS improvement methodology for specific organizations; the Health Equity Studio that operationalizes the Equity pillar's disparity analysis for specific patient populations; the AI Analyst that retrieves and synthesizes HTR intelligence on any topic in real time.

The Advisory services described in Chapter 16 are the institutional bridge between the book's frameworks and specific organizational decisions. Every concept in this book exists at the intersection of understanding and doing — grasping the framework clearly

enough to explain it, and translating the framework into a specific decision, program, contract, or investment in a specific organizational context. The book handles the first challenge; the Advisory services handle the second.

A Note on the AHS Restructuring Chapter

Chapter 11 — The AHS Restructuring Roadmap — occupies a unique position in this book. It is simultaneously the most analytically integrated chapter (bringing all six pillars to bear on a single organizational design problem) and the most operationally specific (written for the AHS staff, Health Care Delivery Advisory Committee members, Vermont legislators, and hospital leaders who must execute the Statewide Strategic Plan mandate).

If you are a Vermont practitioner, Chapter 11 is written for you directly. It is designed to be usable as a working document — a framework you can bring to a planning meeting, a governance structure you can adapt for your organization, a metrics framework you can reference when building the Strategic Plan's accountability architecture.

If you are not a Vermont practitioner, Chapter 11 is still important reading, because the organizational design questions it addresses — how does a state agency restructure from program administration to population health system operation, how do you align a multi-program organization around geographic service areas, how do you build the analytics capacity for strategic decision-making when that capacity does not currently exist — are questions every state will face. Vermont's answers may not be directly transferable, but the analytical framework for thinking through the answers is.

What This Book Cannot Do

Intellectual honesty requires stating what this book cannot do, alongside what it can.

This book cannot predict whether Vermont's transformation will succeed. The statutory mandates are in place. The capital is flowing. The organizational machinery is in motion. But the outcome depends on thousands of decisions that will be made by hospital executives, primary care physicians, community health workers, legislators, regulators, and patients over the next five to ten years. This book provides the analytical framework for those decisions; it cannot make them.

This book cannot substitute for the operational expertise required to execute transformation. The frameworks in this book are accurate and the evidence base is solid, but implementing a global budget requires actuarial expertise, regulatory knowledge, and operational experience that a book cannot provide. That is what Advisory services, technical assistance organizations, and experienced practitioners are for.

This book cannot resolve the political contests that surround healthcare transformation. Reference-based pricing is opposed by hospital systems that benefit from high prices. Global budgets are feared by hospitals that worry about access restrictions. AHS restructuring is resisted by programs that do not want to be reorganized. These are legitimate conflicts of interest, and they will not be resolved by analytical frameworks. They will be resolved by political processes — legislative deliberation, regulatory proceedings, stakeholder negotiations — that this book can inform but cannot replace.

What this book can do: provide the analytical architecture for understanding what is happening and why; supply the evidence base for evaluating what is working and what is not; develop the conceptual vocabulary for communicating clearly across the disciplines — clinical, financial, policy, technological, operational — that transformation requires; and give Vermont practitioners the framework to see their specific work in the context of a coherent transformation agenda. That is enough to be worth writing, and worth reading.

Sources for Chapters 1-3: Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024); Vermont Agency of Human Services, Vermont Rural Health Transformation Program Application (November 2025); Vermont RHT Program Activity Details (December 2025); GMCB VHCURES Data and Analytics documentation; KFF State Health Facts (Vermont); Vermont Department of Health; Act 68 of 2025; Act 167 of 2022; AHEAD State Agreement (January 2025); all sources cited in subsequent chapters.